A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

29 April 2026

Dr Kerryn Gijsbers
East Brunswick Medical Centre

Dear Dr Gijsbers,

Re: Dr Emily Pilkington
DOB: 11/06/1992 Phone: 0404 238 765
Address: Unit 416/270 Lygon Street, BRUNSWICK EAST, VIC, 3057
Email: ehpilkington@gmail.com
Medicare: 3487847124 / 1

Summary

- **Small hiatus hernia and incidental histological finding of eosinophilic oesophagitis.**
- **Sessile serrated adenoma identified**
- **Rectal inflammation (proctitis) of uncertain significance, lacking features of chronicity.**
- **Commencement of mesalazine suppositories and weaning of pantoprazole.**
- **Surveillance gastroscopy and colonoscopy recommended in three years.**
- **No action required by you.**

I had the pleasure of conducting a review with Emily following her recent gastroscopy and colonoscopy.

Gastroscopy revealed a small hiatus hernia, which likely predisposes her to reflux, although the oesophageal mucosa appeared macroscopically normal. Interestingly, biopsies of the oesophagus demonstrated an eosinophil count meeting the diagnostic criteria for eosinophilic oesophagitis. As Emily does not currently experience dysphagia or food bolus obstruction, I have advised her that this is an incidental finding. No specific treatment is warranted at this stage, but we will monitor for the development of any obstructive symptoms. Biopsies of the stomach and small intestine were unremarkable, excluding Helicobacter pylori and celiac disease.

The colonoscopy identified a sessile serrated adenoma. While this was successfully removed, these lesions are recognised as having higher premalignant potential compared to standard tubular adenomas. Consequently, I have recommended a surveillance interval of three years for her next colonoscopy.

Regarding her lower gastrointestinal symptoms, the terminal ileum and colon were histologically normal. However, biopsies from the rectum showed evidence of inflammation. This proctitis lacked the chronic architectural changes typically required to definitively diagnose ulcerative colitis. The aetiology remains uncertain and could represent early-stage inflammatory bowel disease or be related to mechanical factors. Given her clinical history, we have opted for a cautious management approach.

Impression and Plan

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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I have initiated a trial of mesalazine suppositories to be used nightly for the next two months. We will then repeat a faecal calprotectin test to see if the inflammation has resolved. Emily is also awaiting an intestinal ultrasound at the hospital, which I will follow up on, to further exclude isolated small bowel Crohn's disease.

We discussed her medications, and she will now begin a gradual weaning programme from her pantoprazole to avoid acid rebound hypersecretion. She may use Gaviscon liquid or antacids as required during this transition. I have reassured her that she may continue to consume alcohol in moderation. I plan to review Emily in a few months to discuss the ultrasound results, the repeat calprotectin level, and the clinical response to the mesalazine suppositories.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

Investigation Results:

PILKINGTON, EMILY HELEN : MEH 908865, SVP 908865

DOB: 11/06/1992 Age: 33 years

Unit 416, 270 Lygon St, Brunswick East 3057

Results from 11/04/2026 ..... Episode# 26P275584 Test Hist (Histology - Routine)

FINAL ANATOMICAL PATHOLOGY AAnat

Lab #: 2026B009405 **CLINICAL NOTES** Altered BH. Increased calprotectin. Duo, gas, oesophagus, TI, random colon = N. Rectum =? Proctitis vs N. Ascending polyp = SSA. **MACROSCOPIC DESCRIPTION** 1. Specimen labelled "Oesophagus bx". Three mucosal fragments measuring 1-5 mm. A1. 2. Specimen labelled "Gastric bx". Two mucosal fragments measuring 4-7 mm. A1. 3. Specimen labelled "Duodenum bx". Five mucosal fragments measuring 1-3 mm. A1. 4. Specimen labelled "TI bx". Two mucosal fragments measuring 3-4 mm. A1. 5. Specimen labelled "Ascending Colon Polyp". Multiple polypoidal mucosal fragments measuring 5 x 4 x 2 mm in aggregate. A1. Please note: Associated vegetable matter not processed. 6. Specimen labelled "Random Colon bx". Two mucosal fragments measuring 4-10 mm. A1. 7. Specimen labelled "Rectal bx". Five mucosal fragments measuring less than 1-4 mm. A1. Due to small nature of specimen, it may not survive processing. **MICROSCOPIC DESCRIPTION** 1. The sections show stratified squamous mucosa with mild to moderate basal cell hyperplasia in one fragment as well as intercellular oedema. There are also up to 30 intraepithelial eosinophils within the fragment showing basal cell hyperplasia. No neutrophils or areas of ulceration are seen. There is no metaplasia, dysplasia or malignancy. 2. The sections show transitional gastric mucosa with maintained foveolar architecture. There is an increase in chronic inflammatory cells within the lamina propria, including scattered plasma cells. No active or granulomatous inflammation is seen and Helicobacter pylori-like organisms are not identified. There is no intestinal metaplasia, dysplasia or malignancy. 3. The sections show duodenal mucosa with Brunner's glands

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and maintained villous to crypt ratios. There is a normal complement of chronic inflammatory cells within the lamina propria. There is no increase in intraepithelial lymphocytes at villous tips. No active inflammation, granulomata or organisms are identified. There is no dysplasia or malignancy. 4. The sections show small bowel mucosa with maintained villous to crypt ratios. There is a normal complement of chronic inflammatory cells within the lamina propria. There is no increase in intraepithelial lymphocytes at villous tips. No active inflammation, granulomata or organisms are identified. There is no dysplasia or malignancy. 5. The sections of colonic mucosa show areas of serrated luminal architecture with dilated glands extending to crypt bases. There is no dysplasia or malignancy. 6. The sections show colonic mucosa with maintained crypt architecture. There is a normal complement of chronic inflammatory cells in the lamina propria. The subepithelial collagen plate is of normal thickness. No active inflammation or granulomata are seen. There is no dysplasia or malignancy. 7. The sections show colonic mucosa with mostly maintained crypt architecture and areas of increased mixed inflammatory cell infiltrate within the lamina propria including neutrophils which extend into crypt and surface epithelium. No granulomata, crypt abscesses or viral inclusions are seen and there is patchy, focal lamina propria fibrosis in one fragment. Prominent lymphoid follicles with germinal centres containing scattered tingible body macrophages are also seen. The subepithelial collagen plate appears of normal thickness. No granulomata or viral inclusions are seen. There is no dysplasia or malignancy.
PATHOLOGIST'S OPINION 1. Oesophageal biopsy: Squamous mucosa with one fragment showing basal cell hyperplasia and up to 30 intraepithelial eosinophils per high power field. See comment. 2. Gastric biopsy: Mild, inactive chronic gastritis. 3. Duodenal biopsy: Normal duodenal mucosa. 4. Terminal ileum biopsy: Normal small bowel mucosa. 5. Ascending colon polyps: Sessile serrated lesion/polyp without dysplasia. 6. Random colon biopsy: Colonic mucosa within normal limits. 7. Rectal biopsy: Mildly active proctitis. Comment: The oesophageal biopsy showed basal cell hyperplasia and increased intraepithelial eosinophils. If sampling was from the distal oesophagus, changes are likely to be associated with reflux disease, however, eosinophilic oesophagitis cannot be exlcuded. Please correlate clinically. Pathologist: Dr Rohini Joshi

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